

Mon, Jan 09, 2023 5:38AM • 29:54

Interviewer 00:00

And just to start with as well. Do you want to go with sorry, [Name] or [Name]?

Therapist 00:08

Yeah. [Participant],

Therapist 00:10

[Participant]. All right. Perfect. Thanks so much, [Participant] So just to start with as well, I wanted to get a better sense of you're sorry, by the way, do you do prefer if I keep my video on or to keep it off?

Therapist 00:23

Welcome to keep it on. That's fine. Yeah.

Interviewer 00:28

I wanted to get a better sense of your clinical practice. So I just wanted to first clarify what your primary modality of couples therapy is?

Therapist 00:38

My primary modality what?

Interviewer 00:42

Of what couples therapy?

Therapist 00:45

Oh, I would, I guess it would be sort of broadly psychodynamic and, and emotion focused.

Interviewer 00:50

Psychodynamic and emotion focused. How long have you been working as a couple of therapists now?

Therapist 00:58

Well, I, I've been a therapist since [year]. And I've been doing couples therapy all the way along, along with individual therapy and family therapy. I came to the University of Rochester, in [year], to do a fellowship and couple and family therapy, and got training there. And then I came on the faculty, where I've been doing a couple and family therapy in the family and Marriage Clinic, in the Department of Psychiatry at the University of Rochester Medical Center, since [year], after my fellowship, and then I've been teaching the couple of family therapy, rotation to the residents, psychiatry residents since [year]. I don't necessarily think of myself as, as a couples therapist, I'm a therapist that works with couples, I've been known as a family therapist and couples therapist, I get referrals for both. And I now I'm doing a lot of individual work around Mind Body disorders, people with conversion disorder, and that's not couples therapy is something else entirely. And for that I use ISTDP. less information than you were wanting. But that's my whole position in history.

Interviewer 02:28

Interesting. All right. All right. Thank you for that. But of course, for this interview, we'll be focusing on your experience, primarily as in your therapy. That's right. And are there specific types of couples that you provide your services to?

Therapist 02:47

No, no, no, I don't have a specific niche of what kinds of couples or kinds of problems.

Interviewer 02:54

All right. And you provide services to couples in the [Country] primarily? Is that correct?

Therapist 03:00

Entirely? Yes. Entirely.

Interviewer 03:03

All right. And then wanted to ask, I know, there's a broad range of issues here. But when the clients come to you, what is the usual presenting issue? Are there even that the Secret Service?

Therapist 03:15

Oh, they're not getting along? And they're frustrated? Often is the classic communication problems. They'll come in for that. That's the predominant complaint?

Interviewer 03:28

And do you do a formal assessment?

Therapist 03:32

Well, yes, and no, may I do an intake where I find out about the nature of the difficulties, but it isn't straight. It's not a structured assessment, per se. And I don't use any instruments, paper pencil, I don't have any questions to have questionnaires or things. I just talked to them about what's going on what the problem is, what they're hoping to have happen, how long it's been going on, and why they think that's happening, that kinds of..

Interviewer 03:59

Interesting, and you don't use any instruments, why? .

Therapist 04:04

That wasn't part of my training, and I'm I seem to do fine without them. I think there would be things that would be I've often thought I should I should use some instruments, but then I just never got around to it. It's a matter of being busy and not developing that takes take some would it takes some energy to do that. Or go to a workshop or something like that. And, you know, then begin implementing a different way of working.

Interviewer 04:37

And ever since you started your a couple of therapy practice on our on the [Year]. You've never used any of these structured pen and paper assessments?

Therapist 04:49

Correct.

Interviewer 04:49

All right. And I've mentioned this in the email as well, but across the years, different therapists and different researchers have cracked right so concepts are constructive relationship quality quite differently from each other. I wanted to get your view about it for you, what is the relationship quality?

Therapist 05:11

Well, I think it relationship quality is kind of a combination expression of the couples satisfaction with the relationship on the one hand, and how well they problem solve with each other, how free they are to talk about what troubles them and how adept they are at hearing each other's points of view was sort of the psychological mindedness, the mentalization, the better the mentalization, the better the ability to understand where the other person is coming from the better relationship quality.

Interviewer 05:49

Psychological mindedness is the same as perspective taking or that more?

Therapist 05:55

No, I think mentalization is kind of similar to perspective taking psychological mindedness would be like having, having a developed observing ego, being able to observe oneself, think about oneself and one's responses, and understanding why they're happening, not necessarily, because the other person did x, therefore, that's why I do why but that person did x, that reminds me of how I've always treated in this way, historically, and then I tend to overreact these ways. And those overreactions, I realized, aren't helpful, but they're driven by my history, and my interpretation of what the person has said. And then I have painful feelings underneath that, that I don't want to talk about, so I act them out instead. So that's that ability to unpack one's own inner motivations and sort of flow of emotions and thoughts, psychology, mindedness, and liking.

Interviewer 06:52

Interesting. And so for you, what differentiates between couples that have a higher relationship quality, and those with a low relationship quality?

Therapist 07:01

They can self correct is like, high relationship quality couples can get back on course, I mean, if they really high relationship quality, they don't need to come into couples therapy, because they figure this stuff out. They have resilience, they like a self healing tire. You know, when you get a nail puncture, there's this stuff inside the tire seals that puncture hole up, and it doesn't go flat.

Interviewer 07:24

Interesting. And why do you think some people especially when you were saying that, you know, some of the couples don't need to go to therapy? They can self correct? Why do you think some couples do that more easily, in a way, in a sense, and other couples,

Therapist 07:37

I think it's kind of has a lot to do with how one was treated growing up and what you learned about how to how to manage yourself kind of the quality of the quality, the attachment to help the health of the parents and the health of the way the people related to you when you were young. It has a lot to do with your your sense of self and your sense of how to relate and connect with others.

Interviewer 08:02

Would that in your professional experience? Would you say that that's the most important thing in a high quality relationship that's like ability to self correct ability, to self heal the relationship?

Therapist 08:19

Probably Yeah, yeah. And ability to the ability to have empathy for the other person. Well, that gets into perspective taking. And that empathy allows you then to being able to take the other person's point of view, be be aware of your own and work with it too, without getting defensive guards against becoming rigid, and projecting and building walls and cut off and digging in and polarize polarization. So all all those things lead to impasses in the relationship, and those repeated impasses over the years grow into distance and dissatisfaction, hostility, but the ability to do the opposite. And think of perspective, take and reflect, disclose and observe and be non defensive, non judgmental about stuff and other leads to the ability to communicate what's going on for you disclose vulnerable feelings, cultivate empathy,

Interviewer 09:23

Interesting. So this is a particular way of really looking into the relationship right. Psychological mindedness, mentalization of effective dating, etc. I wanted to also excuse as well, that's how you conceptualize the relationship often differ from how your clients conceptualize their relationship? And does this impact therapy? Does this impact the service that you give them?

Therapist 09:49

It can impact, I mean, like couples will often come in to therapy, hostile and angry with one another and aiming at each other, so they look to me to agree with them and blame the spouse that they're angry with. So to the extent that they're dedicated to that position and treat me like a judge, and try to get me to take sides, to that degree, the therapy can bog down when I refuse to do that. But if I'm, if I'm, if I'm having a good day, though, I'll refuse to do that in a way that's engaging to them and helps them feel validated and not threatened. And then they see that there's a different way this possible to go.

Interviewer 10:33

Interesting. So how do how do clients often think about the relationship actually? Then they come in? Like for them? What is the relationship for them? A girl speaking very broadly speaking?

Therapist 10:50

Well, that's awfully abstract, I'm not sure. Knowing how to answer that. In terms of how couples see the relationship. Often, the relationship started off with lots of positive qualities. But those qualities have either deteriorated or negative qualities have intruded over time. So their their comfort in the relationship changes has changed. In the large majority, there's mutual dissatisfaction, sometimes you get one person is quite satisfied, or the other person is quite dissatisfied. But more commonly in the people that I see they're both when they're happy with how things are.

Interviewer 11:38

And talking about that mutual dissatisfaction and sense we know that their relationship is a complex system, right involves love sex, identity, relationship, maintenance, behaviors, context. and responsiveness and satisfaction, all of that, during therapy, how do you decide which one to focus on, or which one in this complex system to target?

Therapist 12:02

Whatever I sense is most prominent in terms of what they're complaining about, or what their, what their, what their difficulties are. And then I just started trying to find out what gets in their way of solving it.

Interviewer 12:18

Is there a usual most prominent thing that they usually start out with?

Therapist 12:25

In terms of what they want? Or?

Interviewer 12:27

Yes or what they're complaining about?

Therapist 12:31

Oh, they're complaining often about being misunderstood and criticized and invalidated and ignored. And usually, one person has that complaint more than the other. But when you start unpacking how the other person is feeling, they feel it too, but their their way of managing it looks different. So typically, the male will just withdraw and get quiet and not need anything, quote, unquote. And the female will be the one who's more aware of emotional distance and more bothered by it. The male often is more comfortable with emotional distance and bothered by too much emotional closeness because they find it dysregulated.

Interviewer 13:11

Interesting, all right. So when do you determine when to end a couple therapy?

Therapist 13:17

When to end it? You mean end it like we've achieved our goals or end it like, Oh, this isn't going to work?

Interviewer 13:25

Both but maybe we can start with the first one?

Therapist 13:28

Well, when they're when they're saying they're feeling satisfied, and things are better. And then say, well, have we achieved what we got? If you got what you came for? And then we mutually

Interviewer 13:41

From start to finish? Would you? Would there be an average of how long it usually takes for your couple clients?

Therapist 13:48

Oh, I would say [1-5] months to [1-5] year.

Interviewer 13:54

[1-5] month to [1-5] year. Of how many sessions? Is that once a week?

Therapist 13:57

20 Yeah, weekly. Well, weekly, weekly, weekly, like 20 to 40 sessions.

Interviewer 14:07

So weekly, one hour every week quantify how long does that usually take?

Therapist 14:13

My Sessions are 50 minutes typically.

Interviewer 14:17

All right. And for the other one you said that you also ended when you know you don't believe

Therapist 14:26

I tend not to tell people this is pointless, we need to stop, I wait. I wait too late aside that. I keep I keep being open in chipping away at it. And they'll get to a point where you know we're not getting anywhere we just keep fighting this isn't working. So they are stopped coming.

Interviewer 14:46

For those types of couples. Why do you think that is?

Therapist 14:51

I tend to blame myself rather than that I blame I mean, my perspective is that if I could figure out what it is that we need to get to help them connect better than then they would, they would be feeling like things are moving along the examined there's exceptions, of course where people just aren't committed or that one of them's having an affair or things like that. But there are there are certainly couples where something that chemistry or my approach is just isn't quite working. So that's a bit of a mystery. So I don't always know why what exactly seems to be going wrong. Other than that, I can feel I can feel it too, that we're not that the session is they get stuck. And then we all get stuck together. And I probably

getting stuck in a similar way to how I was getting stuck with them in a similar way to how they're getting stuck with each other.

Interviewer 15:59

That's a very interesting dynamic. Would you say? For example, like proportion? Can you give like a proportion of the clients? That was a couple of that you have that, that you encounter that issue it?

Therapist 16:11

we're like, we'll get stuck with each other the way they're stuck? 20%?

Interviewer 16:16

Hmm. Interesting, interesting, because these tend to be very intractable problems as long as I can. I can imagine the difficulty and that sort of dynamic.

Therapist 16:27

Right? Now, if you know, if I were if I specialized in couples therapy, and my belief would be that I would have developed more ways of working to understand how to how to avoid those kinds of impasses, or how to handle them better. But because I'm more of a generalist, historically, around couples therapy, I actually almost do more family therapy than couples.. Well, depends, actually, it varies year to year. And individual, so I'm doing all these different things that I don't. For example, I haven't done extra specialty training and couples therapy, the speak of and if I if I was doing like 80% of couples who work and I would go into trainings, I would I'd be much better a couples therapist.

Interviewer 17:12

Would you say? Would you say like, what would be the breakdown between like individual family and couples? For example, this year in your practice?

Therapist 17:21

Oh, this year, it'd be like 95%, individual 2% couple and 3% of family

Interviewer 17:31

Interesting, Was that always the case?

Therapist 17:36

It's been? No, it's been trending that way, based on some changes in responsibilities that I'm some of some of the centers that I work in, have shifted over the years. And so now I'm working in neurology, for the majority of my time, with patients with conversion conversion disorder. Those patients I take up much more heavily individual therapy approach.

Interviewer 18:03

So before you before you can specialize in a conversion or disorder, it was more...

Therapist 18:12

It was like [16-20] years ago, my my sort of therapeutic headquarters was in a marriage and family therapy clinic. And I was doing a lot of teaching on that subject, too. So I was doing a lot of family therapy and a lot of couples therapy [11-15] years ago, but I've shifted over much more heavily into this epilepsy, this conversion disorder work.

Interviewer 18:36

All right. Now, we're currently developing as well, relationship quality scale that hopefully better represents the relationship as a complex system, that aiming to help therapists and treatment planning and evaluation. How interested would you be in in a scale like this?

Therapist 18:55

Me personally, at this point in my career, not not interested at all, mainly because I'm really just sort of leaving couples therapy behind at this point..

Interviewer 19:07

All right. If I know you're not personally interested in that, but for example, like throw yourself [6-10] years ago, or when you were still.

Therapist 19:16

Oh I would be, I would certainly be interested in something like that. Yes.

Interviewer 19:21

But I wanted to ask, like, if you were still, you know, primarily practicing, as a couple therapy, what would you be looking for in a newly developed skin like this? Like, how can we make sure that this scale would have been useful to your practice?

Therapist 19:39

That's an interesting question. I guess it would have. I would, I would want a scale that would give me some perspective on what's going on in the couple that is not easily discoverable by a clinical interview. That it would it would gather the kind of information in an efficient way that would, would help point out something things that that are going on under the hood that are contributing to these impasses that they're in

Interviewer 20:09

Any pointers to what those would be that are not easily discoverable..

Therapist 20:15

No, I'm not a good person to ask because I'm not an I'm not an instrument developer or an instrument user. So I don't have a lot of instrument categories to help you with unfortunately,

[Portion Redacted]

Interviewer 22:56

All right. And just so I've got, again, imagine what your what your answers would be. But again, to ask, Would you be interested in further collaborating for the development of the scale?

Therapist 23:09

I don't think it would be very helpful.

Interviewer 23:12

All right, no problem at all. Actually, we are close to wrapping up. So before we wrap up, I know it's been a brief session, but I would like to just go over the things that we have been saying over the session. To start, I wanted to get a better sense of your clinical practice, you said that you broadly are psychodynamic in your modality as well as emotion Focused Therapy started out in your individual therapy practice actually in [Year]. And then over the years, trained for or got the skills for couples therapy. And when clients come to you what usually the presenting issue is not getting along or and other communication problems. And then you said that you don't do a formal assessment you do like you do an intake interview, about the nature of their conflicts about their treatment goals about the length of standard desert going on, etc. Not necessarily structured and you don't use instruments, but this isn't on any sort of dislike on them on your part, or principle dislike against them for you hasn't really been part of the training. And it just really takes you know, energy to pick techniques up more workshops, etc. And that's why and over the, over the course of your career as a couple therapist, and you're no more a couple of therapists, you really haven't been using these types of structured assessment, or pen and paper assessment. And when asked about relationship quality for your release combination of couples satisfaction, and how well they problem solve, through including what you were saying about the psychological mind and now it's about mentalization and perspective taking that you know they are thinking about themselves there, they're aware of what's happening about them, they're already aware of the sorts of effects of the other grids on on them, etc, their ability to unpack the study. And for you, what differentiates couples with higher relationship quality and those lower relationship quality is is sort of the relationship can self correct, they can go back on ?, they have a sort of resilience, your birth or like a self healing tire, almost. And this includes the ? skills on characteristics of empathy and perspective taking, again, which goes against being rigid about making walls and taller, they polarizing the couple, which often leads to an impasse. So for you, you said that, why other couples are, you know, can can more easily do this is just really the quality of the attachment growing up the health of how they have been related to other relationships growing up, as well. And we were talking about the difference of the conceptualization of relationship between you and your clients. What you were talking about here, that's that couples usually come to therapy or hostile and angry, and they may and trying to pull you in as their like, ally of trying to get you on their side, which also, of course, impacts the way that you do the therapy. And when I ask how you target, you know, how do you focus on which one to prioritize and therapy, for you really is what's most prominent, what they're most complaining about. And usually, it's this issue relating to them feeling No, right, feeling validated, etc. And then when you determined to end the couple therapy, it's when they're already feeling satisfied, or when they're feeling better already, which usually take [6-10] to [11-15] months of weekly 50 minute sessions. But of course, you also mentioned the cases where you don't feel like it's moving along, where you get stuck along with them with you know, feeling stuck with the with the negative patterns that they they also exhibit in their relationship. So you, of course, you're transitioning out of couple and

family therapy. So you are not truly interested or not really interested in the scale that we're developing, although what you said that, you know, it would be really useful for a scale like this to be to get a perspective to to uncover things from the public couple that are not easily discoverable in a clinical interview. So finding something, something under the hood, so to speak. And then you said that this would be more, I think, appealing to Gottman style, garden school type of therapist, rather than psychodynamic dynamic therapists who really is an instrument based not an instrument based practice. So, yes, is there anything you would like to add to that?

Therapist 28:09

No, you covered it very well, you took good notes.

Interviewer 28:13

All right. And now before we end, I would just like to talk to you about how we will handle the data. Of course, with a recent, you know, open science directors we want this to we want this project to be as open and as verifiable as ethically possible. So I wanted to go over with you some of the data sharing option that some of the data sharing option and please let me know which one you're more comfortable with. One option is that transcript portions can be shared, none of which can be identified to you. The other option, if you were bolder, is that the whole transcript removed of sensitive and overly identifying details. So it will be anonymized. So the whole transcript anonymized can be shared. Although in this option, there is a minimal chance that someone who knows you professionally well can piece together the details in the transcript and narrow it down to who [name] say believe it is, although again, no sensitive information to share, but this option will allow more context for potential readers of the research project. So of the two Which one will you be more comfortable with?

Therapist 29:29

Either one is fine.

Interviewer 29:31

Either one. Alright, so I will put you on the transcript for now I will be sending this to one of you. And if you want anything redacted and then just let me know.

Therapist 29:47

Okay. All right.

Interviewer 29:48

Thank you so much before the and actually wait, let me stop the recording because this is not technically part of it and more